جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : منيرة السيد هاشم محمد
المحافظة : القاهره **الحي / المدينة :** دار السلام **العنوان :** 0
رقم البطاقة : 27911052301887 **رقم التليفون :** 01288064588 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 05-11-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: منيرة السيد هاشم محمد
National ID: 27911052301887

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

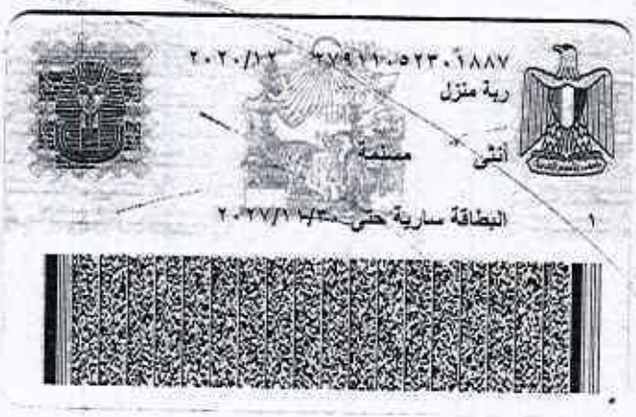
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

صحة مصر

06/30/2026

MDT FORM

Name:	Mounira Elsaid Hashim Mohamed	**MRN:**	4328
Age:	47 Y	**ID:**	27911052301887
Menopausal Sta	pre	**Phone:**	01028165544
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	free	**Reason Of MDT:**	post op

Clinical Data:

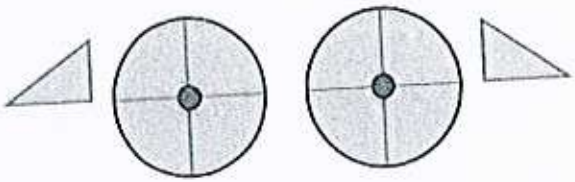
BX: IDC G2

ER: 8 **PR:** 8 **HER2:** 3 **KI67:** 50

Sonomammography:

Mets Workup:

R L



TNM

History:

POST NACT MAMOGRAM ACR C
2 INSERTED CLIPS
FAINT GROUPS OF MICRO CALCIFICATIONS ARE SEEN IN THE UO AND LOQ
COMPLETE RADIOLOGICAL REPONSE WITH NO DEFINITE RESIDUAL MASS
NO DEFINITE PATHOLOGICAL AXILLARY NODES ARE SEEN IN TODAY'S STUDY

LT SM + SLNB (post NACT)
no residual remnants of previously invasive tumor
free margins
SLNB 0/3
yp T 0 yp N 0 (SLN)
for possibility of adj ttt

Recommendations:

MDT Outcome & Plan

cT3N0 --> NACT TCHP 6 Lt SM + SLNB complete PCR
For adj Herceptin (complete 1y)
PORT
ET Zoladex + Nolvadex 5y

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د / نانسي حسن					

Name: منيره السيد هاشم محمد **MR. :** 4328	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has left-sided breast cancer with estrogen-receptor (ER) score 8/8, progesterone-receptor (PR) score 8/8, and HER2 immunohistochemistry 3+. Neoadjuvant chemotherapy (NACT) has been completed, and the patient is awaiting surgical approval; therefore, phesgo is being continued today as cycle 7. The patient has completed 7 cycles of phesgo to date.

Multidisciplinary team discussion on 30 June 2026 staged the disease as clinical T3N0. The patient received neoadjuvant TCHP chemotherapy for 6 cycles, followed by left-sided mastectomy and sentinel lymph node biopsy, achieving a pathologic complete response (PCR). Post-operative management includes adjuvant Herceptin (completed 1 year), radiotherapy (PORT), and endocrine therapy with Zoladex plus Nolvadex for 5 years, together with tamoxifen, Zoladex, and supportive care every 3 months. The patient has completed 6 cycles of the TCHP neoadjuvant regimen.

The patient is currently on tamoxifen and supportive care every 3 months while awaiting Herceptin approval; the next follow-up is scheduled for 11 November 2026.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

Patient Name	: الأستاذه / منوره السيد هاشم محمد	Request No.	: 13006465
Patient No.	: 27911052301887	Request Date	: 11/05/2026
Path No.	: 1455/26	Verification Date	: 01/06/2026
Sex	: Female	Financial Category	: جرستاف روسي - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of Specimen: Left simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:
* Left simple mastectomy specimen measured 20x15x4 cm, with covering skin ellipse measured 15x13 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site 6x4 cm, located 0.7 cm from the least deep margin.
* Sentinel lymph nodes: Fibrofatty tissue piece entangling 3 lymph nodes.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:
* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Microcalcification: Present.
* Nipple & areola: Free.
* Deep margin: Free.
* Sentinel lymph nodes: Three lymph nodes free of metastatic deposits (0/3) with therapy effect.
* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left breast(Post-NACT), simple mastectomy & SLNB:
- No residual viable remnants of the previously diagnosed invasive tumor.
- Deep margin: Free.
- Sentinel lymph nodes: Three lymph nodes free of metastatic deposits, LNs (0/3) with therapy effect.

Verified by :
Dr Maha Guimie

غرفة رقم (4) – الدور الأول – مبني رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / منيره السيد هاشم محمد	Request No.	: 13006465
Patient No.	: 27911052301887	Request Date	: 11/05/2026
Path No.	: 1455/26	Verification Date	: 01/06/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- Stage: ypT0, ypN0 (sln).

Signature
Dr. Walaa Ibrahem Ghoneim.

Verified by :
Dr Maha Guimie

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

جوستاف روسي
انترناشونال

ECHO REPORT

NAME : منيره السيد هاشم DATE: ١٥ / ١١/2025

ID NUMBER : ٢٠٤٢/ ٢٥ SEX : ٤ AGE: ٤٦

EF	64%	55-75%	PWT	٠.٨	0.6-1.1cm
LVEDD	٤.٨	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.7	2-4cm	LA	3.7	2-4cm
FS	28%	25-45%	RV		0.7-2.3com(supine)
SWT	٠.٩	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RWMA
LA and Ao Root Dimensions Normal
Mitral Valve N
Aortic Valve
Tricuspid Valve trivial TR
Pulmonary Valve
Right Side and Systolic Function TAPSE: 2.3
Intracardiac Masses and Effusion

DR. محمد الخطيب

Echo
[[DOCTYPE:ECHO]]

Patient ID: 86416	Patient Name English: Monirah Alsid Hashem
Age: ٧٥ سنة ٨ شهر	2025/07/28 12:16:38 PM :Report Date

Left Ultrasound guided clip insertion

Indication:

Left breast:

- Under complete aseptic condition & ultrasound guidance two clip insertion was done for 2 breast upper central and inner malignant mass lesions at 11-12 o'clock, the clips was inserted at the center of the lesion and this was confirmed by mammographic CC & ML

Best Regards,

Dr. Rania Mohamed

Ultrasound
[[DOCTYPE:ULTRASOUND]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Monira Al Sayed Hasham Mohamed	Date : 22/12/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 2042/2025	Pat. ID: 133443

FOLLOW UP LEFT BREAST HIGH RESOLUTION ULTRASOUND

INDICATION:
Left breast pathologically proven malignant mass, treated by NACT, for assessment

REPORT:

LEFT BREAST:

- The ill-defined solid mass located at 12 o'clock is no more seen in today's study.
- No other cystic or solid lesion seen.
- No definite pathological axillary node is , only one level I node is seen with cortical thickness reaching 3.8mm.
- Free infra and supra-clavicular regions.

OPINION:

- Complete sonographic response of the left neoplastic process, (BIRADS 6).

Best regards,
Dr. Mahi Mokhtar

Ultrasound
[[DOCTYPE:ULTRASOUND]]

ECA

Contrast enhanced mammography

- **Indication:** Left breast lump.

Findings:

Low energy images:

- Heterogeneously dense glandular parenchyma of both breast (ACR c)
- Left central focal asymmetry with distortion.
- No microcalcific clusters detected.

Recombined contrast enhanced images:

- Bilateral nodular background parenchymal enhancement
- Left upper inner spiculated heterogeneously enhancing mass lesion is seen measuring about (1.6 x 2.1) . Associated anterior heterogeneous non mass enhancement showing regional distribution measuring about (5.4 x 2.8 cm). the overall distance measures about (7.4 x 3.2 cm). it measures about 0.7 cm from the nipple
- No suspicious enhancing lesions or areas of pathological non-mass enhancement detected in the right breast.

Opinion:-

- Left upper inner pathologically enhancing mass with anterior non mass enhancement as described (BIRADS 5) for histopathological confirmation.(Biospy was done in the same setting)
- No right breast enhancing masses or non mass enhancement.

Thanks for referral,

Dr Safaa Ibrahim. , MD

Dr. Mennatallah M.Hanafy. MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

03/16/2026

MDT FORM

Name:	Monira Mohamed Hashim Mohamed	**MRN:**	4328
Age:	46 Y	**ID:**	27911052301887
Menopausal Sta		**Phone:**	01288064588
NRS SCORE:		**Marital Status:**	M
BMI:		**Family History:**	+VE
CO Morbidity:	FREE	**Reason Of MDT:**	POST NACT

Clinical Data:

BX: IDC G2

ER: 8 **PR:** 8 **HER2:** 3 **KI67:** 50

Sonomammography:

POST NACT MAMOGRAM ACR C
2 INSERTED CLIPS
FAINT GROUPS OF MICRO CALCIFICATIONS ARE SEEN IN THE UO AND LOQ
COMPLETE RADIOLOGICAL REPONSE WITH NO DEFINITE RESIDUAL MASS
NO DEFINITE PATHOLOGICAL AXILLARY NODES ARE SEEN IN TODAY'S STUDY

Mets Workup:

BONE SCAN FREE

History:

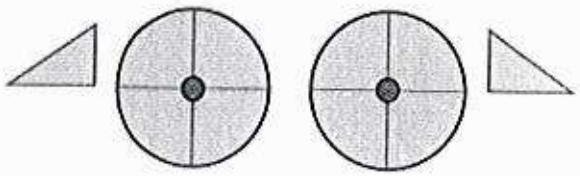
Recommendations:

FOR BCS AFTER WIRE LOCALIZATION

MDT Outcome & Plan

FOR BCS+SLNB AFTER WIRE LOCALIZATION

R L

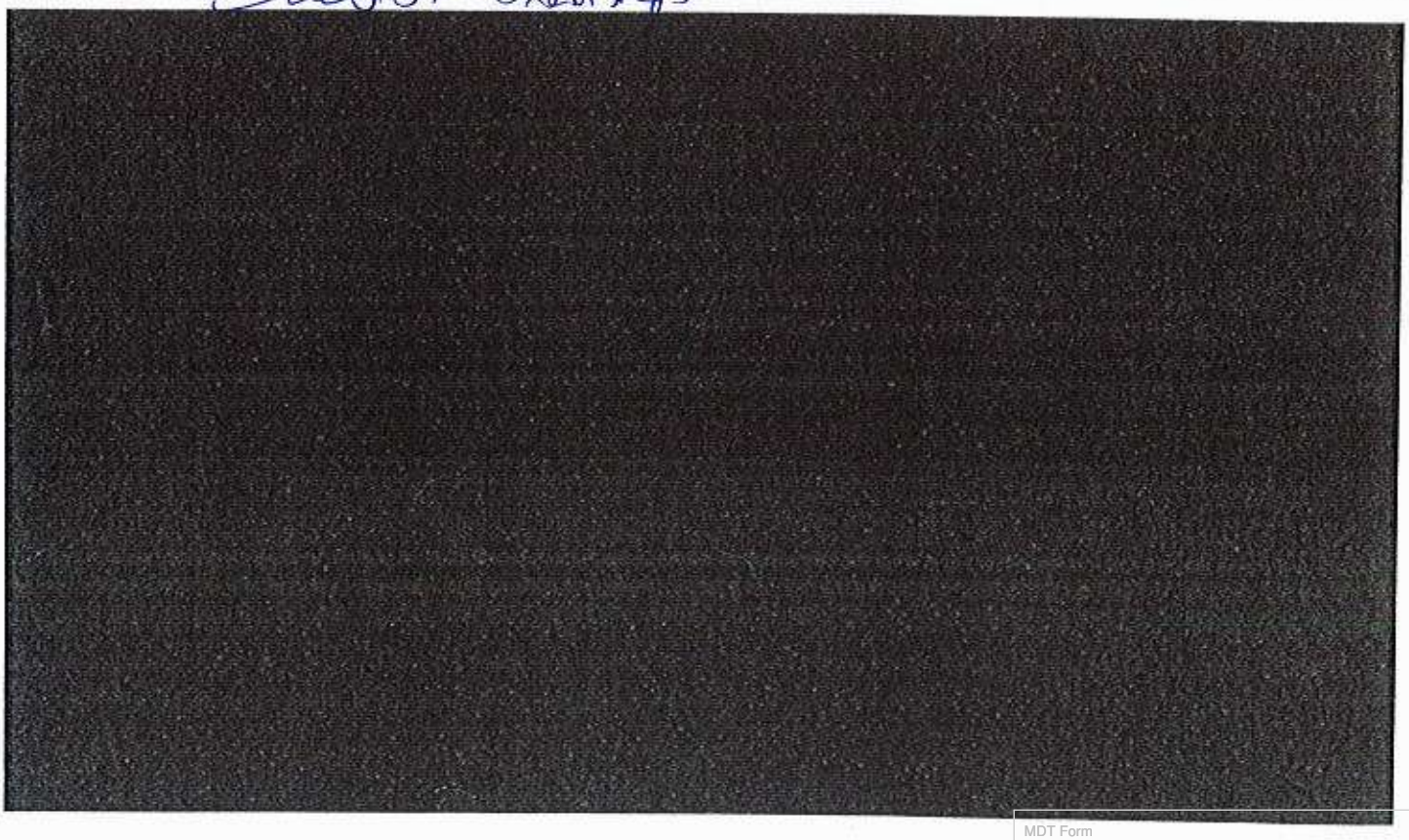


TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
	د/ ترمين مصطفي	د/ خالد الكيدي			

د/ هبة احمد



MDT Form
[[DOCTYPE:MDT_FORM]]

Name: منيره السيد هاشم	Date: 6/22/2025
Age: 46 Years	ID: 27911052301887

Bilateral mammography with complementary High resolution Ultrasound Examination

Indication: Left breast lump.

Findings:

- Heterogeneously dense glandular parenchyma of both breast (ACR c)
- Upper central focal asymmetry with distortion. On complementary US it shows a solitary spiculated mass lesion with surrounding desmoplastic reaction and casting posterior acoustic shadowing, it is seen at 12 o'clock about 5 cm from the nipple measuring about 2.5 x 1.4 cm in maximum dimensions.
- No clustered microcalcifications detected.
- No right solid or cystic lesions.
- Normal contour and skin thickening of both breast.
- Normal nipple /areola complexes.
- Normal duct caliber.
- Bilateral axillary non-specific lymph nodes.
- No supra-clavicular lymph nodes.

Opinion :-

- **Left upper central 12 o'clock suspicious solid mass lesion (BIRADS 4), for histopathological correlation and CESM.**
- **Right breast fibroadenosis.**

Thanks for referral,

Dr.Eman Badawy,MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name: moneera elsayed hashem	Date: 24/06/2025
Age:	ID: 27911052301887

Contrast enhanced mammography

- ***Indication:*** *Left breast lump.*

Findings:

Low energy images:

- *Heterogeneously dense glandular parenchyma of both breast (ACR c)*
- *Left central focal asymmetry with distortion.*
- *No microcalcific clusters detected.*

Recombined contrast enhanced images:

- *Bilateral nodular background parenchymal enhancement*
- *Left upper inner spiculated heterogeneously enhancing mass lesion is seen measuring about (1.6 x 2.1) . Associated anterior heterogeneous non mass enhancement showing regional distribution measuring about (5.4 x 2.8 cm). the overall distance measures about (7.4 x 3.2 cm). it measures about 0.7 cm from the nipple*
- *No suspicious enhancing lesions or areas of pathological non-mass enhancement detected in the right breast.*

Opinion:-

- *Left upper inner pathologically enhancing mass with anterior non mass enhancement as described (BIRADS 5) for histopathological confirmation.(Biospy was done in the same setting)*
- *No right breast enhancing masses or non mass enhancement.*

Thanks for referral,
Dr Safaa Ibrahim. MD
Dr. Mennatallah M.Hanafy, MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]